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Mental health and social trust: Age and religiosity as moderators among young people in Croatia and Slovenia

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This study explored the moderating effects of age and religiosity on the relationship between social trust and mental health among young people aged 16–29 in Croatia ($N = 1{,}177$) and Slovenia ($N = 1{,}227$). Mental health was evaluated using two distinct measures: the Burnout Assessment Tool (BAT) and the Depression Anxiety Stress Scale (DASS-9). Social trust was assessed as trust in close family members and generalised trust. Religiosity was measured by religious service attendance, the importance of God in daily life, and support received from religious organisations. The findings indicate that age moderates the relationship between trust in close family members and BAT in both countries, and the relationship between trust in close family and DASS in Croatia. In terms of religiosity, all three dimensions significantly moderated the relationship between generalised trust and DASS in Croatia, while only one of them did so when BAT was used as the dependent variable. No significant moderation effects of religiosity were found in Slovenia. In summary, the results suggest that the negative psychological impact of low social trust can be mitigated through various mechanisms. The moderating effect of age implies that psychosocial maturation during adolescence may buffer the impact of low trust in close family on mental health. Religiosity, on the other hand, may act as an emotional substitute for generalised trust, especially in highly religious contexts. Additionally, the study highlights the importance of the chosen mental health indicator, as moderating effects varied depending on whether BAT or DASS-9 was applied.

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1. Introduction

This study aims to clarify the relation between trust, mental health, young people and individual religiosity using empirical data from older adolescents and emerging adults in Croatia and Slovenia. Most broadly, social trust refers to the belief in the reliability, truth, or competence of an individual or group to fulfil a promised or expected action

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(Hardin, 2002) and is fundamental to both interpersonal relationships and subjective well-being (Simpson, 2007). A rich body of literature has explored the role of trust on various aspects of subjective well-being and mental health, along with the numerous moderators and mediators influencing this relationship. Although this research is accumulating, much is left to be empirically investigated in terms of different aspects of mental health in particular, the moderators of the relationship between mental health and trust, as well as nuances in the cross-cultural variation of these relationships.

Research focused specifically on the relationship between young people's mental health and social trust highlights a significant gap in understanding the contextual variations of this relationship. Generally, trust is widely recognised as a positive factor contributing to the well-being of young people, with trust in family being particularly strongly linked to the well-being, psychosocial functioning and mental health of children and adolescents (Tuominen & Haanpää, 2022). It plays a vital role in fostering healthy attachment, which is fundamental to their psychosocial functioning and subjective well-being (Fett et al., 2016; Grzegorzewska & Farnicka, 2016; Raja et al., 1992). Even trust in unknown people seems to be a protective factor against mental health distress (Bai et al., 2019; Martínez et al., 2019). However, much less is known about the moderators of this relationship, particularly how the relationship differs across developmental stages. While some research suggests that trust increases with age and has a stronger positive impact on the well-being of older individuals compared to younger ones (Poulin & Haase, 2015), it is still largely unclear how different types of trust (for example, generalised trust compared to particularised trust) relate to mental health outcomes across different age groups. Additionally, it is uncertain whether any factors might mitigate the negative effects of low trust on mental health. These questions are particularly underexplored during the vulnerable developmental period of emerging adulthood. Furthermore, there is limited understanding of whether these relationships vary across different cultural contexts. This paper aims to contribute to the ongoing debate about the relationship between trust and mental health across different contexts by addressing three key gaps in the existing literature.

Firstly, it remains unclear how the positive effects of trust on mental health differ between age groups and the type of trust under investigation. Do adolescents benefit from trust more or less than older adults? Is this dependent on the type of trust under investigation? Some research indicates this might be the case, but very few empirical studies have illuminated these age differences, and none to our knowledge have investigated the role of different types of trust in this context.

Secondly, for those young people who experience low trust, can the negative effect of this low trust on their mental health be buffered by any other variables? There is some reason to speculate this effect might be less pronounced if other protective factors are in place, like individual religiosity or level of social support.

And thirdly, it is unclear whether these effects differ between different cultural contexts. The only study that specifically investigated the moderating role of individual religiosity on the relation between young people and mental health found that religiosity buffers adolescents against the psychological effects of low social trust, but their study was limited to the UK (Tsomokos & Dunbar, 2023). There are many reasons to expect cultural context, including societal norms, the role of religion in social cohesion, secularisation levels, and trust dynamics, to significantly shape the relationship between trust and mental health and the role of individual religiosity in this relationship.

In order to empirically test how trust and mental health are related in young people and how this relationship differs based on age, type of trust, for individual religiosity, and cultural context, we use data from a recent survey in which we measured trust in family members and generalised trust, depression and burnout.

The remainder of the paper is structured as follows. First, we lay out a theoretical framework to clarify how the concepts relate to one another and review the literature on the current state of understanding of these relationships. Then, we formulate hypotheses to theorise how the two types of trust relate to mental health of adolescents and emerging adults in relation to their religiosity and broader cultural context. This is followed by the methods and data section. The results section reports the findings from our analyses, using data from a recent international project on the well-being and support structures of young people in Southeast Europe. We conclude by discussing the main implications of our results.

2.1. Expected relations between trust and mental health of young people

General agreement exists that trust can be understood as the expectation that others will act in ways that support the well-being of an individual or group or, at the very least, avoid causing harm (Offe, 1999). Beyond this broad definition, the extensive literature on trust explores a wide range of conceptual frameworks, distinctions, and typologies (e.g. Cook et al., 2009; Hardin, 2002; Lane, 1998; Uslaner, 2015; Yamagishi & Yamagishi, 1994). A particularly valuable distinction frequently highlighted in this body of work is that between generalised trust and particularised trust. In this paper, we will specifically address generalised trust and include trust in family members as a representative example of particularised trust.

2.1.1. Generalised trust and mental health

Generalised trust refers to the trust in strangers or most people in society, which reflects the individual's belief that most people can be trusted (Stolle, 2002). It is commonly measured by the survey question, 'Generally speaking, do you believe that most people can be trusted or that you can't be too careful in dealing with people?' (Uslaner, 2015). Generalised trust is characterised by its stability and independence from specific individual or group traits, making it abstract and rooted in broader societal norms and institutions rather than personal experiences. For generalised trust, positive associations have been found with happiness, life satisfaction, subjective health (Adedeji et al., 2023; Helliwell & Wang, 2010; Kuroki, 2011; Usher, 2007; Yamamura et al., 2015) and higher well-being across the entire lifespan (Poulin & Haase, 2015), while low trust is associated with more depressive symptoms (Adjaye-Gbewonyo et al., 2018; Fermin et al., 2022), greater perceived health-related threat and worry (Thoresen et al., 2021), and even higher all-cause mortality (Miething et al., 2020) and likelihood of suicidal ideation (Yamamura, 2015). Studies done specifically on young people have found similar findings, with general trust being positively associated with a sense of self-worth and quality of social relationships Li et al. (2023), psychosocial adjustment (Betts et al., 2017; Rotenberg et al., 2021), and overall mental well-being (Haugstvedt, 2023) of young adults. Generalised social trust, then, is directly and strongly linked to subjective well-being and mental health (for a detailed review see Helliwell & Wang, 2010; for

the most recent meta-analysis see Zhao et al., 2024). The relationship is so strong and consistent that some consider social trust to be a health-promoting personality trait (Kawachi, 2018).

2.1.2. Particularised trust and mental health

A distinct and critical form of trust in adolescence is particularised trust, which pertains to relationships with individuals in a young person's immediate environment, such as family members, teachers, and peers (Flanagan & Stout, 2010) with whom one shares a direct personal connection (Kim & Kim, 2021). Foundational developmental theories, such as Bowlby's (1969) attachment theory and Erikson's (1963) theory of psychosocial stages, emphasize the essential role of building interpersonal trust early in life for adaptive psychosocial functioning later on (Simpson, 2007). During adolescence, sustaining trusting interpersonal relationships becomes more challenging due to expanding social networks (Lee et al., 2016) and increasing independence from parents (Van den Bos et al., 2012). This period of life is already marked by vulnerability to mental health issues, such as anxiety and depression (McLaughlin & King, 2015). The ability to form and maintain trusting relationships plays a crucial role in helping adolescents navigate these challenges successfully (Van den Bos et al., 2012). Therefore, fostering trust during adolescence is essential, as it lays the foundation for well-being, mental health and adaptive psychosocial functioning in adulthood.

To deepen our understanding of how trust relates to young people's mental health, it is helpful to consider the established model of trust beliefs, which conceptualises trust as comprising three key dimensions: reliability, emotional trust, and honesty (Rotenberg, 2010). Reliability refers to the expectation that others will keep their word, emotional trust involves confidence that others will avoid causing harm, such as by maintaining confidentiality, and honesty reflects the belief that others will act truthfully and with benevolent intent. Trust, then, is tied to the strength of one's conviction that others consistently demonstrate these qualities. In this sense, this framework particularly highlights the significance of interpersonal trust for adolescents. When young people perceive those around them as unreliable, emotionally harmful, or dishonest, they may withdraw socially. This withdrawal can hinder their ability to develop critical skills and resources, such as social support, peer relationships, close bonds, and academic success. Therefore, fostering a strong sense of trust is vital for their social, emotional, and developmental outcomes.

A large body of evidence supports this idea. Specifically, research indicates that maladaptive interpersonal trust beliefs are associated with lower peer acceptance and greater aggression, social nonengagement, peer rejection, loneliness, depressive symptoms, and anxiety (Betts et al., 2013, 2017; Rotenberg, 1994; Rotenberg et al., 2005; Rotenberg & Boulton, 2013). The presence of trusted relationships, on the other hand, can even serve as a protective factor against the onset of mental illness (Grzegorzewska & Farnicka, 2016). This is particularly relevant for adolescents as longitudinal studies have shown that maladaptive attachment relationships in infancy lead to particular risk for social maladaptation during adolescence (Doyle & Cicchetti, 2017). Moreover, trust early in life has been prospectively positively linked with later psychosocial functioning in adolescents (Malti et al., 2013, 2016), healthy relationships (Simpson, 2007), academic achievement, and social competence (Wentzel, 1991), and negatively with depressive symptoms (Lester & Gatto, 1990). Trust in family members specifically predicts health seeking behaviours

and plays a valuable role in addressing stigma and rejection associated with illness (Lau et al., 2020). More broadly, parental trust has been consistently linked to better psychosocial adjustment in adolescents (Sugimura et al., 2020). Similarly, peer trust has been shown to also positively predict psychosocial adjustment (Betts et al., 2013; Rotenberg et al., 2005), reduce loneliness (Rotenberg et al., 2004) and foster better peer relationships in children (Rotenberg & Boulton, 2013). Interpersonal trust is even negatively associated with suicide ideation via perceived burdensomeness and thwarted belongingness in adolescents (Hill et al., 2019). In a recent review, trust has been shown to be negatively related to mental illness in adolescents (Clarke et al., 2021; Fett et al., 2016) and positively to willingness to seek professional help (Corry & Leavey, 2017; Reardon et al., 2017).

2.2. Expected moderators of the relationship between mental health and different types of trust

We base our study on the core assumption that the impact of trust on the mental health of young people is different across two main moderators, age and religiosity. We proceed with a brief discussion of the theoretical underpinnings of this assumption and a review of the current evidence for it.

2.2.1. Age as a moderator of the relationship between trust in family trust and mental health

While trust seems to positively contribute to well-being and mental health across the lifespan, both theoretical frameworks and empirical evidence suggest that its effect may vary across different age groups. Our reasoning is based on life-course theory, which posits that individuals' social networks, priorities, and social roles shift over different life stages (Clark & Eisenstein, 2013; Elder, 1994; Poulin & Haase, 2015) and in the research that supports the notion that trust levels differ developmentally across life stages (Harbaugh et al., 2003; Sutter & Kocher, 2003, 2007). Researchers have long recognised that trust begins developing in early childhood and plays a crucial role in fostering adaptive psychosocial functioning (Erikson, 1963), enabling children to explore the world and form relationships with people beyond the family unit (Bowlby, 1969; Erikson, 1963; Rotter, 1980). After that, adolescence and young adulthood represent particularly critical periods for social and psychological development, since this is a period in which the majority of mental health problems emerge (Solmi et al., 2022). It is also a period of notable shifts in interpersonal trust (Sijtsma et al., 2023). This underscores the importance of a more detailed understanding of the role of different forms of trust in the mental health of young people. As they age, young people increasingly report more trusting and communicative relationships with peers rather than parents, and in general people beyond the family unit and begin to fulfil attachment needs (Nickerson & Nagle, 2005). Similarly, communication and affection towards parents tend to diminish from early adolescence through emerging adulthood (Parra et al., 2015), a distinct life stage spanning ages 18–29 that serves as a transitional period between adolescence and full adulthood and is marked by identity exploration in areas such as careers, relationships, and personal values, alongside the gradual assumption of adult roles and responsibilities (Arnett et al., 2014).

This gives us reason to believe that the extent to which trust impacts psychosocial outcomes depends on the type of trust and the type of psychological outcome. In the transition to adulthood, other forms of relationships are likely beginning to impact well-being more. As individuals age, they tend to extend their trust beyond immediate family to a broader social circle (Li & Fung, 2013), reflecting a developmental trend where growing independence and broader social interactions lead to a diversification of trust from family-centric to more generalised forms. As young individuals mature, they become less reliant on close family members. As a result, the negative effects of lacking trust in these familial relationships on psychological well-being become less pronounced with age. This suggests that trust in family plays a stronger role in mental health during earlier stages of life and gradually weakens as young people gain greater independence and transition into adulthood.

Based on this, we hypothesise that adolescent mental health is impacted by trust in family members more than emerging adults in their twenties. Specifically, we posit that age moderates the association between trust in family and mental health so that the protective effect of trust on mental health is larger for adolescents than for older individuals in the transition to adulthood. If that were the case, this might help us understand why generalised trust and even trust in friends only have a limited effect on adolescents' mental health. While trust in peers is already increasing during adolescence, perhaps its positive effect becomes more apparent later on, while during adolescence family still plays a crucial role. This might be because during adolescence, peers can also negatively affect well-being (McMahon et al., 2020), so family can serve as a protective factor. Based on these theoretical assumptions, we formulated our first hypothesis (H1) according to which the positive impact of trust in close family members on mental health decreases as respondents' age increases.

2.2.2. Religiosity as a moderator and the role of cultural context

The second moderator we are interested in is religiosity. We examine this relationship at the individual level and explore whether this moderation effect varies between our two study countries, Slovenia and Croatia, considering their differing levels of religiosity at the national level. The notion that the effect of generalised trust depends on individuals' level of religiosity is based on the following reasoning.

There are two competing perspectives that attempt to explain the foundations of generalised trust. The experiential perspective argues that social trust is relatively fragile and subject to environmental influences throughout life, whereas the cultural perspective views trust as a stable trait formed early in life (Dawson, 2019). Drawing on the experiential perspective, we propose that the impact of trust on mental health is not fixed but varies based on individual characteristics. Specifically, we hypothesise that generalised trust has a weaker effect on mental health for individuals with higher levels of religiosity. We also posit this relationship may not be uniform and is likely influenced by specific contextual or demographic factors that act as moderators, which we explore in the next subsection.

Religiosity has been consistently associated with better psychological outcomes. Comprehensive reviews (Lucchetti et al., 2021; Papaleontiou-Louca, 2021) and meta-analyses (Garssen et al., 2021; Hoogeveen et al., 2023; VanderWeele, 2021; Yaden et al., 2022) offer strong evidence of this relationship. In transition to adulthood specifically, established

patterns of religious coping have been shown to mitigate distress (Eliassen et al., 2005). Religiosity is also a moderator of the relationship between perceived stress and psychological adjustment (Reutter & Bigatti, 2014) and between social isolation and psychological well-being (Abolfathi Momtaz et al., 2011). Similarly, a recent study (Haugstvedt, 2023) found that social support moderates the negative effect a lack of trust in others has on young people's mental health. This suggests that the ability to cope with life struggles young people face might be weakened if they are less trusting of others but also that this negative effect is less detrimental if they receive adequate social support, for example, provided by their religiosity. This is understandable considering the finding that the quality of social connections is essential for the maintenance of trust, which is built on a base of shared positive experience and nurtured by continued connections (Helliwell & Wang, 2010). In fact, psychosocial mechanisms have been shown to drive the association between trust and health (Miething et al., 2020). This can be extended to suggest that religiosity may serve a moderating role, particularly because it is linked to greater social support. Together, these findings highlight the potential of religiosity as an effective coping resource, especially for young people.

While religiosity is not identical to social support, it is closely linked to it at both the individual (Merino, 2014) and national level (Kaasa, 2013). At the individual level, studies have shown that religious participation provides access to social support, which in turn enhances well-being. Religious involvement offers social support, learning to give and receive forgiveness, and engagement in community activities that bolster social capital and community health (Koenig, 2023). Spiritual support from church members seems to provide people with more positive religious coping responses (Krause et al., 2001). At the national level, research suggests that in more religious societies, the link between social trust and well-being is stronger due to the prevalent social support systems embedded within religious communities. Trust tends to be higher in more religiously pluralistic national contexts, while religiosity in these contexts helps individuals maintain good health (Upenieks & Orfanidis, 2022). In general, then, religion deeply affects and shapes social support processes in close relationships (Merino, 2014). Based on this, we hypothesise that the effect of generalised trust on young people's mental health is moderated by individuals' level of religiosity, so that the association is weaker for those who are more religious. Our second hypothesis (H2), therefore, posits that the positive impact of generalised trust on mental health decreases as religiosity increases. In short, we hypothesise this because religious belief and community can serve a similar psychological and social role as trust, making trust less critical for those with strong religious ties. Conversely, for less religious individuals, generalised trust plays a more central role in supporting mental health.

As mentioned, the influence of religion on mental health and potentially also on the relationship between social trust and mental health is shaped not only by individual characteristics but also by the wider cultural context. For example, in highly religious European regions, service attendance is related to less depressive symptoms in highly religious areas more strongly than in less religious ones (Van de Velde et al., 2017). Similarly, national religiosity acts as a buffer against difficult life circumstances and amplifies the effects of personal religiosity on subjective well-being (Tay et al., 2014). In countries where religious socialisation is common, spiritual practice positively correlates with well-being, whereas the reverse is true in less religious nations, further underscoring

the need to consider cultural variability in conceptualising religion's psychological effects (Lun & Bond, 2013).

Specifically, cultural differences have been noted to influence the religiosity – well-being relationship, particularly for young people (Jensen, 2021). For example, Calandri and Graziano (2022) suggest that the positive link between religion's importance and adolescent well-being is stronger in Italy, where Catholicism is deeply embedded in the culture, than in the U.S. Additionally, the association between individual social capital and life satisfaction depends on national social capital, suggesting that the benefits of social trust are influenced by how well individuals fit within their social environments (Elgar et al., 2011; Poortinga, 2006; Subramanian et al., 2002). While these studies do not specifically focus on trust, they suggest that religiosity may have the potential to mitigate the negative psychological effects of low social trust on mental health. Thus, for individuals with low generalised trust, the negative effect on mental health may be mitigated by their individual religiosity, but only in contexts where religion plays a significant cultural role.

Building on the evidence that religiosity might buffer the negative effects of low social trust, it is crucial to examine whether this relationship holds across different cultural contexts. To explore these variations, we propose a comparative study of Croatia and Slovenia to examine whether cultural differences influence the moderating role of religiosity in the relationship between social trust and mental health of young people. Understanding these cultural nuances is essential for assessing the generalizability of our findings across societies (Trommsdorff & Chen, 2012). Croatia and Slovenia offer a compelling comparison due to their shared historical and linguistic roots as former Austro-Hungarian territories and Yugoslav republics, alongside notable differences in religiosity and socialisation practices. While both share South Slavic linguistic ties and similar demographic profiles (Deliso, 2020), their contrasting roles and prevalence of religion highlight distinct cultural contexts shaping social and psychological phenomena. We provide a brief overview of the key differences.

The most striking difference in the cultural contexts of Croatian and Slovene youth lies in their religious affiliation and participation. In Slovenia, 50% of youth in 2024 report that they do not belong to any religious organisation and never or almost never attend religious services, while this figure is notably lower in Croatia, at 33% (Hasanovic et al., 2024). Conversely, regular attendance at religious services, defined as attending at least once per month, is higher among Croatian youth (26%) compared to only 12% of Slovene youth. This disparity is echoed in the general population, with only 5% of Croats identifying as atheists or nonbelievers (Gavrilović, 2023). The Catholic Church plays a central role in Croatian public life, shaping cultural norms and collective identity, while Slovenia's more secular orientation features a higher proportion of individuals without religious affiliation (Bojinović Fenko et al., 2023). These differences provide a meaningful context for examining the effects of religiosity on youth outcomes. Religiosity fosters social-emotional resilience and personal growth in adolescents, offering a vital coping mechanism (Filipović & Rihtar, 2023), and so Croatia's higher religiosity may provide a robust social framework, offering shared values and support systems that enhance young people's mental health and trust. Conversely, Slovenia's secular orientation may encourage greater personal autonomy but offer fewer religion-based support networks, affecting how young people navigate challenges and develop trust.

Secondly, Croatia and Slovenia also differ significantly in social trust and socialisation practices. Generalised trust is lower in Croatia than in Slovenia, a difference linked to their divergent post-independence trajectories, with Slovenia viewed as more institutionally stable and socially cohesive (Kjærsgård & Anhøj, 2013). Slovenia's secular orientation fosters individualism, with trust rooted in personal interactions rather than community structures (Delhey & Newton, 2005; Halman & Sieben, 2023). In contrast, Croatia's strong Catholic tradition emphasizes collective identity, community-based trust, and solidarity, which have been shown to foster interpersonal trust and support networks that enhance well-being (Bojinović Fenko et al., 2023; Lim & Putnam, 2010; Norris & Inglehart, 2011; Putnam, 2001).

Considering these cultural differences, we posit that individual religiosity will more strongly moderate the relationship between generalised trust and mental health in the more religious Croatia compared to the less religious Slovenia.

2.2.3. The aim of our study

This paper addresses several key gaps in the literature on trust and the mental health of young people. While trust is widely linked to improved well-being and mental health in children and adults, studies focusing on young people are scarce. Recent research highlights the importance of this relationship, with findings indicating that higher trust in adolescence reduces depression and anxiety in early adulthood (Låftman et al., 2024; Tsomokos & Dunbar, 2023). Additionally, the role of moderation effects in this relationship remains underexplored. While macro-level factors such as societal mobility and cultural individualism have been shown to moderate the trust-well-being link (Guo et al., 2022; Wang & Wai Li, 2020), little is known about individual-level moderators, which this paper seeks to address. Furthermore, we aim to expand understanding of the cultural dynamics underlying the relationships between trust, mental health, and religiosity among young people, examining how these interactions vary across contexts. To provide a comprehensive analysis, we utilise two measures of trust, three aspects of religiosity, and two indicators of mental distress, offering a more nuanced exploration of these phenomena.

3. Data and methodology

3.1. Data and analytic strategy

The data analysed in this paper was collected through an online survey developed as part of the research project 'YO-VID22: Well-Being and Support Structures of Young People Before, During, and After the COVID-19 Pandemic' (J7-4597). The survey was administered by the research agency Valicon, Participants were residents of Croatia and Slovenia, aged 16–29. In each country, a quota sampling method was employed to ensure a representative distribution across key demographic and regional characteristics. Sampling was conducted by Valicon, using their online panel 'JaZnam'. Participants were primarily recruited from this existing online panel, but to reach the target sample size of 1,200 respondents per country, additional participants were recruited through panel referrals and targeted social media advertisements. Quotas were established based on national census data and demographic projections for individuals aged 16–29 years, ensuring representativeness across gender, age groups, geographic regions, and education levels.

The analyses in this paper were adjusted using country-specific weights. The weighting procedure was designed to maximise the representativeness of the data by accounting for key demographic and regional variables. Specifically, weights were calculated using a combination of sex-age, region and settlement size, and education level. The weights were computed using the RIM (Random Iterative Method) procedure, which adjusts the distribution of control variables to match the structure of the target population. This method simultaneously weights all characteristics while minimising distortion for each variable, ensuring an optimal balance. The RIM procedure calculates weights through iterative regression analyses, guided by two parameters: (1) a limit that defines the acceptable deviation from target values, and (2) the number of iterations to converge on the desired weights. This approach ensures that the data is appropriately adjusted to reflect the population structure and is suitable for robust modelling.

The Croatian sample consisted of 1,216 individuals (51.2% male; $M_{age} = 22.77$, SD = 3.89), while the Slovenian sample included 1,287 individuals (52.8% male; $M_{age} = 22.87$, SD = 4.00). In Croatia, 28.9% of respondents reported living in a rural settlement or village, 12.4% in a more rural than urban area, 15.8% in a more urban than rural area, and 42.9% in an urban settlement. In Slovenia, 42% of respondents reported living in a rural settlement or village, 20.4% in a more rural than urban area, 17.3% in a more urban than rural area, and 20.4% in an urban settlement. In the Slovene sample, 26.6% of participants were aged 16–19 years, 35.1% were aged 20–24 years, and 38.3% were aged 25–29 years. Similarly, in the Croatian sample, 26.3% of participants were aged 16–19 years, 36.4% were aged 20–24 years, and 37.3% were aged 25–29 years. Young Slovenes reported a significantly lower mean importance of God (M = 3.9, SD = 3.1) than Croatian youth (M = 6.1, SD = 3.4). Similarly, the Croatian sample reported higher mean attendance at religious rituals (M = 3.3, SD = 1.9) compared to Slovene youth (M = 2.6, SD = 1.7). Furthermore, Croatian youth were more likely to receive help from a religious organisation during the pandemic (M = 1.9, SD = 1.2) than Slovene youth (M = 1.5, SD = 0.9). Additional information regarding the sample is provided in Tables 1 and 2.

For the purposes of the present paper, we utilised only a subset of the broader dataset, focusing on variables pertinent to our research objectives. Specifically, we extracted data related to the *Anxiety, Depression, and Stress Scale* (DASS), *Burnout Assessment Tool* (BAT), generalised social trust, trust in close family members, and three dimensions of religiosity. In addition to these variables, we included key sociodemographic variables, described below as covariates.

3.2. Operationalisation and measures

The entire survey instrument included a comprehensive set of questions designed to assess the well-being of young people across multiple dimensions. Data collection was conducted in the fall of 2023 using the Computer-Assisted Web Interviewing (CAWI) method. Participants completed an online survey, providing responses directly via a web-based platform. Average completion time was 28 min per survey.

3.2.1. Mental health

As outlined above, various psychological outcomes have been examined in relation to the impact of trust on young people, ranging from clinical measures of mental disorders and

Table 1. Means, standard deviations, and correlations between the study variables for the Croatian subsample.

Valid N (listwise) = 1177	M	SD	1	2	3	4	5	6	7	8	9	10	11	12
1 Gender														
2 Age	22.77	3.89	.036											
3 Meduc	3.81	1.40	−.071*	−.147***										
4 Feduc	3.64	1.33	−.030	−.103***	.463***									
5 Fin. situation	3.77	0.87	.011	−.110***	.123***	.154***								
6 Urbanicity	2.73	1.28	−.072*	−.010	.216***	.204***	.003							
7 Gen. trust	4.38	2.56	−.135***	−.006	.090**	.107***	.051	.015						
8 Fam. trust	4.50	0.82	−.003	−.007	.022	.016	.101***	−.024	.105***					
9 Rituals	3.28	1.94	−.009	−.015	.007	.010	.011	−.125***	.142***	.091**				
10 God	6.13	3.43	.022	.041	−.061*	−.043	−.002	−.167***	.036	.144***	.692***			
11 Help	1.88	1.21	−.013	.034	.010	−.022	−.031	−.062*	.092**	.012	.490***	.550***		
12 DASS	.039	.93	.146***	−.025	−.026	−.034	−.149***	−.011	−.150***	−.294***	−.018	.028	.061*	
12 BAT	−.019	.97	.212***	−.008	−.012	−.020	−.167***	.018	−.185***	−.245***	−.103***	−.081**	−.048	.729***

Meduc. = mother's educational level; Feduc. = father's educational level; Fin. situation = household's financial situation; Gen. trust = generalised trust; Fam. trust = trust in close family members; Rituals = attendance of religious rituals; God = importance of God in one's life; Help = receiving help from a religious organisation during the pandemic.

** p < .05; **p < .01. ***p < .001.*

Table 2. Means, standard deviations, and correlations between the study variables for the Slovenian subsample.

Valid N (listwise) = 1227	M	SD	1	2	3	4	5	6	7	8	9	10	11	12	13
Gender															
Age	22,87	4,00	.006												
Meduc	4,14	1,49	−.104***	−.154***											
Feduc	3,83	1,44	−.062*	−.095***	.523***										
Fin. situation	3,77	0,86	−.031	−.076**	.192***	.207***									
Urbanicity	2,16	1,18	−.056*	.068*	.128***	.181***	−.039								
Gen. trust	4,36	2,41	−.067*	−.030	.126***	.133***	.080**	−.033							
Fam. trust	4,47	0,85	−.032	.065*	.042	.043	.134***	−.002	.068*						
God	4,00	3,14	−.052	−.042	−.061*	−.064*	−.066*	−.092***	.022	.002					
Rituals	2,65	1,69	.013	−.071*	−.019	−.033	.076**	−.225***	.100***	.016	.644***				
Help	1,54	0,98	−.055*	−.085**	.033	−.009	−.057*	−.085**	.079**	−.054	.480***	.453***			
DASS	−0,04	0,95	.173***	−.050	−.067*	−.042	−.173***	.067*	−.166***	−.230***	.007	−.049	.060*		
BAT	0,02	0,95	.220***	−.041	−.042	−.032	−.145***	.066*	−.182***	−.225***	−.039	−.080**	−.009	.726***	

Meduc. = mother's educational level; Feduc. = father's educational level; Fin. situation = household's financial situation; Gen. trust = generalised trust; Fam. trust = trust in close family members; Rituals = attendance of religious rituals; God = importance of God in one's life; Help = receiving help from a religious organisation during the pandemic.
*$p < .05$; **$p < .01$. ***$p < .001$.

suicidality to indicators of positive mental health and subjective well-being. In this paper, we do not capture positive mental health or subjective well-being (e.g. life satisfaction, happiness, psychological well-being, or positive affect), nor do we incorporate clinical measures of mental illness. We focus on two psychological outcomes considered here as key indicators of low mental health in young people.

3.2.1.1. Self-reported depression, anxiety, and stress. First, we measure non-clinical symptoms of depression, anxiety, and stress, which frequently co-occur (Dobson et al., 2021; Garber & Weersing, 2010) and are major contributors to the disease burden among young people (Klaufus et al., 2022). To capture these interconnected states, we employ the Depression Anxiety Stress Scale (DASS), an established instrument in both clinical and research settings (Dwight et al., 2024; Jovanović, 2024; Szabo & Lovibond, 2022). Specifically, we use the DASS-9, a short form of the original 42-item scale empirically derived through Confirmatory Factor Analysis (Yusoff, 2013). Participants were asked to indicate how often they experienced the described feelings over the past week. Responses were recorded on a 4-point Likert scale, ranging from 0 (*Did not apply to me at all*) to 3 (*Applied to me very much or most of the time*). This instrument captures a broad spectrum of psychological distress symptoms and is specifically designed to assess emotional states over a one-week period, balancing the need to capture recent fluctuations while minimising the influence of momentary mood changes. The DASS-9 has demonstrated strong psychometric properties, with a Cronbach's alpha of 0.72 in a sample of 704 medical school applicants (Yusoff, 2013). For this study, we translated the English version into Slovene and Croatian. The measure demonstrated good internal consistency, with Cronbach's alpha values of 0.87 for the Croatian sample and 0.89 for the Slovenian sample. Measurement invariance tests were conducted for the DASS-9, conceptualised as a one-dimensional construct. Confirmatory factor analyses (CFA) were performed to evaluate the fit of the single-factor model across two countries. For the DASS-9 scale, the Slovenian data indicated excellent model fit (CFI = 0.996, TLI = 0.995, SRMR = 0.035), with factor loadings ranging from 0.607 to 0.761. Similarly, the Croatian data demonstrated strong model fit (CFI = 0.993, TLI = 0.992, SRMR = 0.037), with factor loadings between 0.588 and 0.752. All fit indices exceeded conventional thresholds for good model fit (CFI/TLI > 0.95, SRMR < 0.08), confirming the robustness of the single-factor structure for the DASS-9 across the two cultural contexts.

3.2.1.2. Self-reported burnout syndrome. The second aspect of mental health we examine is burnout, a phenomenon gaining increased attention in adolescence and emerging adulthood. Burnout is a syndrome defined by the combination of inability, reflected in a lack of energy, and unwillingness, manifested as resistance, reduced commitment, disinterest, and disengagement from work tasks, hindering effective task completion (Schaufeli et al., 2020). While traditionally studied in professional contexts (Schaufeli et al., 2020), it is increasingly recognised as a significant issue among students. Burnout in adolescents can have serious mental health consequences, warranting further research (Walburg, 2014). To our knowledge, no research has yet investigated the role of trust in burnout among young people. To evaluate this, we use the 12-item Burnout Assessment Tool (BAT-12), which measures burnout as a prolonged state of physical and emotional exhaustion resulting from chronic stress. The BAT assesses core symptoms

such as exhaustion and psychological detachment, as well as secondary effects like emotional and cognitive dysfunction (W. Schaufeli & De Witte, 2023). This scale is a robust instrument with adequate psychometric properties for measuring individual burnout levels (De Beer et al., 2022). Cronbach's alpha for the BAT-12 was 0.92 for Croatia as well as for Slovenia, reflecting excellent internal consistency. Measurement invariance tests were also conducted for the BAT, conceptualised as a one-dimensional construct. Slovenian data exhibited excellent fit indices (CFI = 0.998, TLI = 0.998, SRMR = 0.032), with factor loadings ranging from 0.783 to 0.902. The Croatian data also showed strong model fit (CFI = 0.988, TLI = 0.987, SRMR = 0.056), with factor loadings between 0.625 and 0.775. All fit indices exceeded conventional thresholds for good model fit (CFI/TLI > 0.95, SRMR < 0.08), confirming the robustness of the single-factor structure for the BAT across the two cultural contexts.

3.2.2. Trust in family members and generalised trust

3.2.2.1. Generalised social trust. Respondents were asked 'Generally speaking, would you say that most people can be trusted or that you can't be too careful in dealing with people?', a standard question for measuring generalised trust. They answered on a scale from 1 ('You can't be too careful') to 10 ('Most people can be trusted').

3.2.2.2. Trust in close family members. Respondents were additionally asked 'On average, to what degree do you trust immediate family members (mother/father, sister/brother, wife/husband, partner)?' They answered on a scale from 1 to 5, where 1 means 'no trust at all' and 5 'very much'.

3.2.3. Religiosity

3.2.3.1. Religious rituals attendance. Participants were asked 'Aside from funerals and weddings, how often do you attend religious rituals?' and answered on the scale from 1 (never or almost never) to 7 (more than once a week).

3.2.3.2. Importance of god. Participants were asked 'How important is God in your life?' and answered on the scale from 1 (not at all important) to 10 (very important).

3.2.3.3. Receiving help from a religious organisation during the pandemic. Participants were asked 'During the pandemic, how much did a religious organisation help you in asserting your rights or solving life problems?' and answered on the scale from 1 (never) to 5 (very often).

3.2.4. Covariates

3.2.4.1. Age. Participants were asked 'How old are you?' within an open-ended question. Age served as a covariate in certain regression models, while in others, it functioned as a moderator.

3.2.4.2. Gender. Participants were asked 'What is your gender?' and answered either Man (1), Woman (2), or Other (3). The third category, consisting of only 0.1% of the entire sample was not included in the regression models.

3.2.4.3. Mother's education and father's education. Participants were asked, 'What is your father's / mother's highest formal education?' with possible responses ranging from 1 (Less than completed primary school) to 8 (PhD). These two variables were used as an indicator of socioeconomic status rather than relying on the participants' own education level. The latter is often closely tied to age, making it a less reliable measure of socioeconomic status among young people.

Household's financial status was used as another dimension of respondents' socioeconomic status. Participants were asked 'What is your financial status?' and chose from options: ranging from 1 (We don't have enough money to cover basic bills and food) to 5 (We can afford everything we need for a good quality of life).

3.2.4.4. Urbanicity. Participants were asked 'How would you describe the place you live?', choosing from the following options: (1) Rural (village), (2) More rural than urban, (3) More urban than rural, and (4) Urban (city).

4. Results

Descriptive statistics and correlations are presented separately for both subsamples in Table 1 (for Croatia) and Table 2 (for Slovenia). In both subsamples, the three variables related to religion were significantly and positively correlated among each other. DASS and BAT scores were also highly correlated with one another. Both generalised trust and trust in close family members were significantly negatively correlated with DASS and BAT scores. DASS is not correlated with attendance at religious rituals in either sample, while BAT shows a negative correlation with attendance in both countries. The importance of God is not significantly correlated with either DASS or BAT in Slovenia and is negatively associated with BAT in Croatia. Receiving assistance from a religious organisation is a positive predictor of higher levels of DASS (but not BAT) in both countries. This correlation contrasts with those observed for the other two measures of religiosity. Age significantly positively predicts trust in close family but not generalised trust in Slovenia and it predicts neither in Croatia. It also negatively predicts religious rituals attendance and receiving help from a religious organisation in Slovenia, while it does not significantly correlated with any of the religious variables in Croatia.

Our basic hypotheses were tested separately for the two dependent variables. The results of multiple linear regression analyses are presented in Tables 3 and 4 (for BAT) and Tables 5 and 6 (for DASS).

4.1. Relation between trust in close family members, mental health and age

Model 1 (see Tables 3–6) reveals a significant negative association between trust in close family and both measures of mental health in both Croatia ($\beta = -0.225$, $p < 0.001$ for BAT and $\beta = -0.282$, $p < 0.001$ for DASS) and Slovenia ($\beta = -0.190$, $p < 0.001$ for BAT and $\beta = -0.191$, $p < 0.001$ for DASS). Our results mostly confirm the first hypothesis (H1) which assumed the positive impact of trust in close family members on mental health decreases as respondents' age increases. Specifically, the relationship between BAT and trust in close family is significantly moderated by age in both countries ($\beta = 0.076$, $p < 0.01$ for Croatia and $\beta = 0.097$, $p < 0.001$ for Slovenia). The moderation is also significant when

Table 3. Standardised coefficients from regression models with BAT as dependent variable, Croatian sample.

	Model 1	Model 2	Model 3	Model 4
Independent variables				
Fam. trust	−0.225***			
Gen. trust		−0.152***	−0.156***	−0.159***
Moderators				
Age	−0.040			
Rituals		−0.056*		
God			−0.104***	
Help				−0.039
Interactions				
Fam. trust × Age	0.076**			
Gen. trust × Rituals		0.046		
Gen. trust × God			0.018	
Gen. trust × Help				0.071*
Controls				
Gender (female)	0.225***	0.207***	0.207***	0.207***
Age		−0.030	−0.024	−0.029
Meduc	0.019	0.025	0.023	0.028
Feduc	−0.005	0.006	0.006	0.004
Fin. situation	−0.138***	−0.160***	−0.163***	−0.163***
Urbanicity	0.026	0.023	0.013	0.027
Adj. R^2	0.127	0.099	0.105	0.101

Meduc. = mother's educational level; Feduc. = father's educational level; Fin. situation = household's financial situation; Gen. trust = generalised trust; Fam. trust = trust in close family members; Rituals = attendance of religious rituals; God = importance of God in one's life; Help = receiving help from a religious organisation during the pandemic
*$p < .05$; **$p < .01$. ***$p < .001$.

Table 4. Standardised coefficients from regression models with BAT as dependent variable, Slovenian sample.

	Model 1	Model 2	Model 3	Model 4
Independent variables				
Fam. trust	−0.190***			
Gen. trust		−0.155***	−0.175***	−0.152***
Moderators				
Age	−0.055*			
Rituals		−0.044		
God			−0.027	
Help				0.010
Interactions				
Fam. trust × Age	0.097***			
Gen. trust × Rituals		0.000		
Gen. trust × God			−0.034	
Gen. trust × Help				0.031
Controls				
Gender (female)	0.227***	0.225***	0.224***	0.226***
Age		−0.072*	−0.072**	−0.068*
Meduc	0.004	0.010	0.009	0.011
Feduc	−0.015	−0.001	−0.002	0.000
Fin. situation	−0.102***	−0.116***	−0.122***	−0.115***
Urbanicity	0.084**	0.069*	0.074**	0.080**
Adj. R^2	0.123	0.102	0.101	0.101

Meduc. = mother's educational level; Feduc. = father's educational level; Fin. situation = household's financial situation; Gen. trust = generalised trust; Fam. trust = trust in close family members; Rituals = attendance of religious rituals; God = importance of God in one's life; Help = receiving help from a religious organisation during the pandemic
* $p < .05$; **$p < .01$. ***$p < .001$.

Table 5. Standardised coefficients from regression models with DASS as dependent variable, Croatian sample.

	Model 1	Model 2	Model 3	Model 4
Independent variables				
Trust in close family	−0.282***			
Gen. trust		−0.141 ***	−0.141***	−0. 145***
Moderators				
Age	−0.059*			
Rituals		0.044		
God			−0.015	
Help				0.067*
Interactions				
Fam. trust × Age	0.094***			
Gen. trust × Rituals		0.071*		
Gen. trust × God			0.059*	
Gen. trust × Help				0.094***
Controls				
Gender (female)	0.149***	0.134***	0.133***	0.134***
Age		−0.047	−0.045	−0.051
Meduc	0.005	0.008	0.010	0.009
Feduc	−0.013	−0.007	−0.006	−0.004
Fin. situation	−0.111***	−0.139***	−0.143***	−0.139***
Urbanicity	−0.004	0.008	−0.002	0.006
Adj. R^2	0.126	0.062	0.058	0.068

Meduc. = mother's educational level; Feduc. = father's educational level; Fin. situation = household's financial situation; Gen. trust = generalised trust; Fam. trust = trust in close family members; Rituals = attendance of religious rituals; God = importance of God in one's life; Help = receiving help from a religious organisation during the pandemic
* $p < .05$; ** $p < .01$. *** $p < .001$.

Table 6. Standardised coefficients from regression models with DASS as dependent variable, Slovenian sample.

	Model 1	Model 2	Model 3	Model 4
Independent variables				
Fam. trust	−0.191***			
Gen. trust		−0.142***	−0.154***	−0.153***
Moderators				
Age	−0.072*			
Rituals		−0.012		
God			0.010	
Help				0.077**
Interactions				
Fam. trust × Age	0.045			
Gen. trust × Rituals		0.010		
Gen. trust × God			−0.016	
Gen. trust × Help				−0.003
Controls				
Gender (female)	0.178***	0.176***	0.177***	0.180***
Age		−0.087**	−0.087**	−0.081**
Meduc	−0.029	−0.022	−0.022	−0.026
Feduc	−0.008	0.003	0.003	0.005
Fin. situation	−0.118***	−0.133***	−0.135***	−0.128***
Urbanicity	0.085**	0.078**	0.080**	0.086**
Adj. R^2	0.103	0.084	0.084	0.090

Meduc. = mother's educational level; Feduc. = father's educational level; Fin. situation = household's financial situation; Gen. trust = generalised trust; Fam. trust = trust in close family members; Rituals = attendance of religious rituals; God = importance of God in one's life; Help = receiving help from a religious organisation during the pandemic
* $p < .05$; ** $p < .01$. *** $p < .001$.

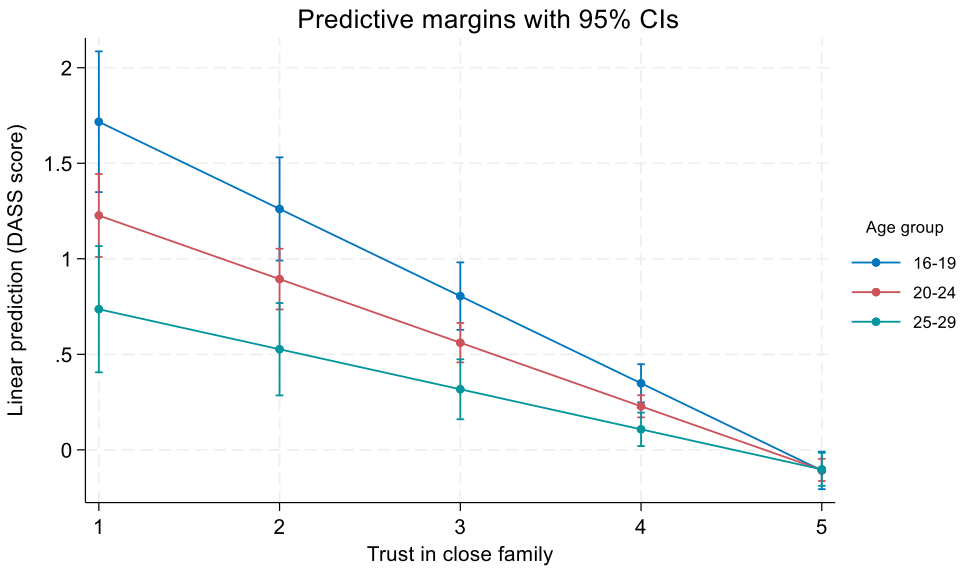

Figure 1. Moderation of age on the relationship between trust in close family members and DASS in Croatia.

the outcome variable is DASS in Croatia ($\beta = 0.094$, $p < 0.001$, see Figure 1) but not in Slovenia ($p > 0.05$). Consistent with our first hypothesis, the value of the interaction terms is positive, suggesting that the negative association between trust in close family members and BAT is weaker (closer to zero) with increasing age in both countries. In these analyses, trust and age were centred around their means, ensuring that the main effects represent relationships at the average level of the moderator, and the interaction term shows how the effect shifts as age deviates from its mean. Because the interaction is positive, it indicates that while higher trust still predicts lower BAT, this protective effect diminishes in magnitude as age increases.

4.2. Relation between generalised trust, mental health and religiosity

Results from Models 2, 3 and 4 are related to our second main hypothesis (H2) that different aspects of religiosity significantly moderate the relationship between generalised trust and mental health, and the third hypothesis that this effect is more pronounced in Croatia than in Slovenia.

Starting with the main effects, we found that generalised trust exhibits a significantly negative association with DASS (see Tables 5 and 6) and BAT (see Tables 3 and 4) in both countries. Religiosity shows a much less consistent association with mental health. DASS is only significantly associated with having received help from a religious organisation during the pandemic (but not with the other two aspects of religiosity), with the association being positive in both countries ($\beta = 0.067$, $p < 0.05$ for Croatia and $\beta = 0.077$, $p < 0.01$ for Slovenia). This means that individuals who received more help were also the ones that were struggling more with their mental health during the pandemic. BAT, on the other hand, is significantly related to the other two

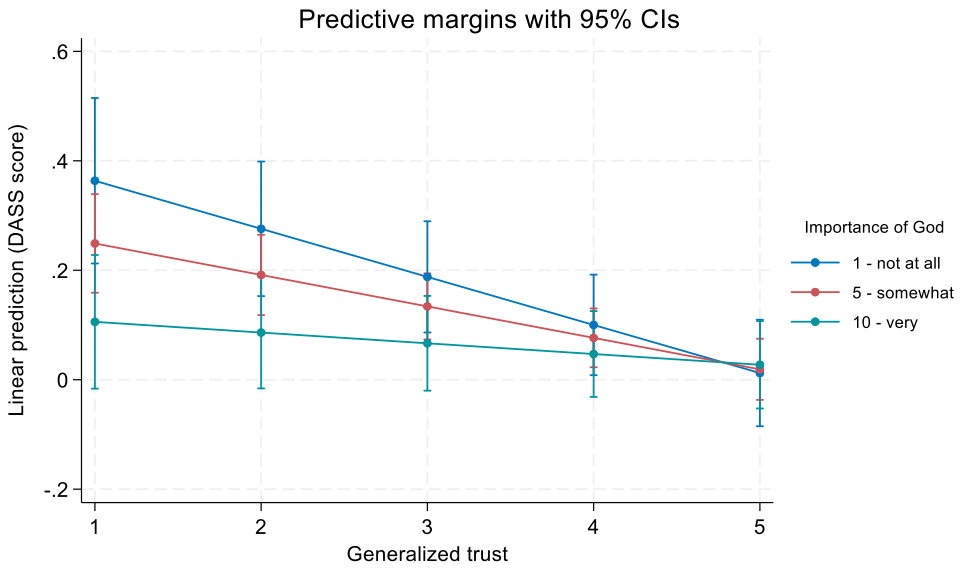

Figure 2. Moderation of importance of God on the relationship between generalised trust and DASS in Croatia.

aspects of religiosity ($\beta = -0.056$, $p < 0.05$ for religious rituals attendance and $\beta = -0.104$, $p < 0.001$ for the importance of God) and only so in Croatia.

Moving to moderation effects, individual religiosity mostly does not moderate the relationship between BAT and generalised trust in either country (see Models 2–4 in Tables 3 and 4), contrary to our second hypothesis. The one exception is having received help from a religious organisation in Croatia which shows that the negative association

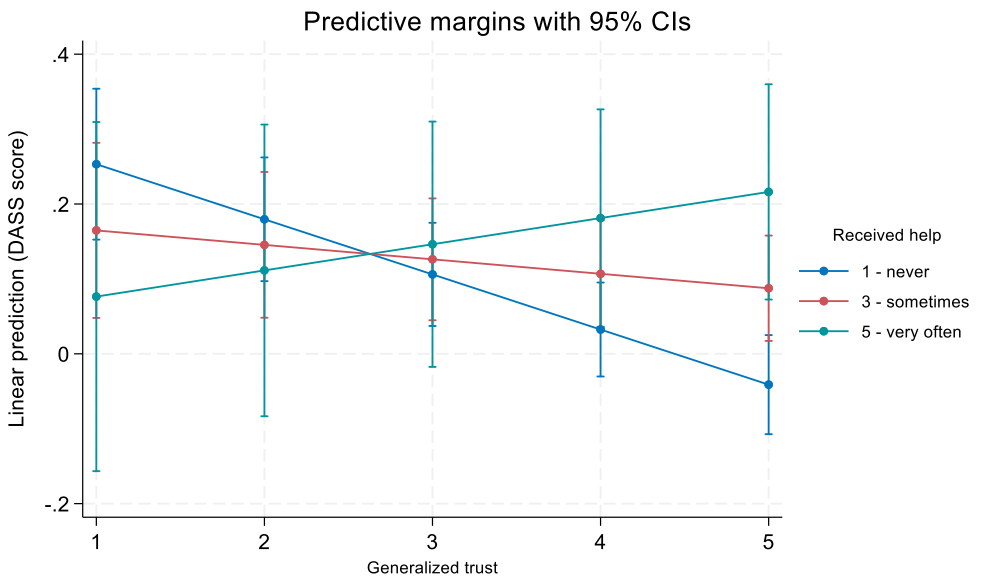

Figure 3. Moderation of receiving help from a religious organisation on the relationship between generalised trust and DASS in Croatia.

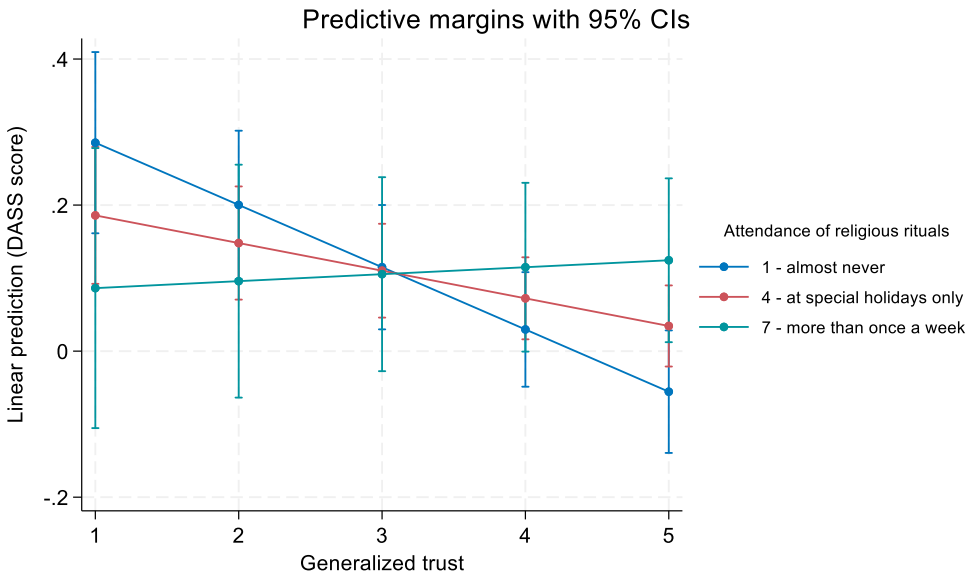

Figure 4. Moderation of attending religious rituals on the relationship between generalised trust and DASS in Croatia.

between generalised trust and burnout was less pronounced for those who received more help, in line with our hypothesis.

On the other hand, all three aspects of religiosity significantly moderate the negative association between DASS and generalised trust in Croatia (Models 2–4 in Table 5 and Figures 2–4). The positive sign of the interaction terms indicates that the negative effect of generalised trust on DASS is less pronounced for individuals that report greater religiosity. Our second hypothesis, then, is confirmed mostly for DASS but not BAT, and for Croatia but not Slovenia. Overall, these results reveal partial support for our second hypothesis, which assumes a moderating role of religiosity on the relationship between generalised trust and young people's mental health.

5. Discussion

This study examined how age and religiosity moderate the relationship between social trust and mental health among young people in Croatia and Slovenia. Using a comparative framework, we sought to explore the interplay of trust, mental health, and cultural context during the transitional period from adolescence to young adulthood.

We first hypothesised that the negative impact of low trust in close family members on mental health would diminish with age. Our results largely support this hypothesis. For burnout, this moderation was observed in both Croatia and Slovenia, while for depression, anxiety, and stress, the moderating effect of age was only significant in Croatia. We can interpret these findings in the following way. Trust in immediate family members remains essential for healthy psychological and social development during adolescence, even as peer relationships grow in importance (Brown & Larson, 2009; Erdley & Day, 2017). Because individuals still heavily depend on their close family members for emotional and social stability to cope with stressors (Helsen et al., 2000), low trust in family has a strong negative association with mental

health outcomes (Guan et al., 2014). As individuals age, they increasingly develop alternative support systems outside their families, such as peer networks, romantic relationships, or institutional supports (Cheng & Chan, 2006; Levitt et al., 2005) and these additional sources of support lessen the reliance on familial trust for maintaining mental health. This shift from family-centred trust to other forms of trust is largely facilitated by social learning processes, particularly the development of cognitive and social skills that enable adolescents to adopt others' perspectives (Kilford et al., 2016). These cognitive advancements allow for the formation of deeper, more complex relationships, highlighting the expanding role of social trust as it extends beyond the immediate family. To summarise, two key conclusions emerge from these findings. First, familial support remains crucial for adolescents, even as peers and romantic partners take on increasing significance in their lives. This underscores the importance of emphasizing the protective role of family trust when addressing adolescent mental health. Second, the results suggest that the critical factor is not necessarily the specific source of social trust, but rather the existence of some form of trust. While the source of trust may shift with age – from family to peers, romantic partners, or other social connections – the essential takeaway is that having trust in a social group is vital for well-being. The encouraging implication is that the specific source of trust is flexible, as long as trust in some meaningful relationship or group is maintained.

Our second main hypothesis proposed that **individual religiosity could buffer against the negative mental health effects of low generalised trust**. This aligns with prior research suggesting a strong connection between religiosity and social support (Merino, 2014), as religious participation often facilitates access to social networks and support systems. These networks can enhance well-being by promoting forgiveness, community involvement, and shared values (Koenig, 2023). Supporting and building on Haugstvedt's (2023) findings that social support can moderate the negative effects of low trust on young people's mental health, our results suggest that religiosity may act as a specific form of such support. Similarly, Krause et al. (2001) found that spiritual support from church members fosters positive religious coping, enhancing resilience in the face of life stressors. Together, these findings highlight religiosity's capacity not only to provide solace and a sense of purpose but also to strengthen social capital and trust within communities. Our findings align with broader theoretical frameworks suggesting that the psychosocial mechanisms driving the relationship between trust and mental health are rooted in the quality of social connections. Trust is built on a foundation of shared positive experiences and is nurtured through continued social interactions (Helliwell & Wang, 2010). Religiosity, as a shared and practiced belief system, provides a structured context for such positive interactions, facilitating the development and maintenance of trust within social networks. This dynamic is particularly significant for individuals with low generalised trust, as religiosity can compensate by offering an alternative source of connection and support. Interpreting our results through these theoretical and empirical lenses suggests that religiosity acts as a compensatory mechanism for individuals with low generalised trust.

Another significant and, to our knowledge, original finding stems from the fact that receiving assistance from a religious organisation (ARO) emerged as a much stronger moderator than the other two measures of religiosity. It is important not to confuse this with the main effect of ARO. Unlike the other two religiosity measures, our results show a positive correlation between ARO and DASS. This can be explained by the likelihood that individuals experiencing greater psychological discomfort are more inclined to

seek, and therefore receive, ARO. However, our focus is not on the main effect, but rather on ARO's moderation effect. Specifically, why would ARO serve as a stronger moderator than more traditional measures of religiosity? We propose that ARO, compared to the importance of God or attendance at religious rituals, involves higher levels of (1) trust in one's religious organisation and (2) direct social support derived from the religious community. Both factors may effectively substitute for the lack of generalised trust. For individuals who distrust others in general, it is psychologically beneficial to place trust (at least) in a religious organisation and to receive direct social support from its members. The fact that this moderating effect was observed predominantly in Croatia points to the importance of cultural context, which we now address in more detail.

As noted, recognising the value of cross-cultural comparison in exploring the interactions between trust, religiosity, and mental health, we examined these hypotheses in two culturally distinct contexts. While we did not make explicit predictions about how cultural differences would influence the findings, we anticipated that examining these relationships across two neighbouring but socioculturally distinct countries would provide valuable insights. In particular, Croatia, characterised by higher levels of religiosity among youth, offered an interesting contrast to Slovenia, where religiosity is lower both at the population level and among young people specifically, which is documented both in existing literature (e.g. Lavrič & Naterer, 2023) and this study's results (see descriptive statistics in the method section). We found clear presence of both main and moderating effect of religiosity in Croatia, whereas almost no such effect was found in Slovenia**.** These findings highlight the crucial role of cultural differences in how religiosity influences mental health. The level of secularisation is likely a key factor in explaining these differences. For instance, previous studies have demonstrated that the positive psychological effects of religiosity tend to be stronger in more religious environments, that is, in national contexts where religiosity is considered normative (e.g. Lavrič & Flere, 2008; Stavrova et al., 2013). Specifically, our finding is in line with research showing that service attendance is related to less depressive symptoms in more religious regions than in less regions (Van de Velde et al., 2017). One of the main reasons for this is likely that religious contexts facilitate the formation of larger and more supportive social networks, which themselves have well-established positive effects on mental health (Barkan & Greenwood, 2003; Bradley, 1995; Ellison & George, 1994; Lim & Putnam, 2010). The positive effects of religiosity on mental health are thus more pronounced in highly religious contexts due to the presence of a stronger religious community and greater opportunities for social support. Conversely, in less religious settings, the positive impact of individual religiosity on mental health may be diminished as individuals have fewer opportunities to connect with others who share their faith, resulting in weaker social ties and less support. These findings underscore the significant differences in religiosity between the two groups and align with expectations based on cultural and religious differences between the two countries. Therefore, it is reasonable to suggest that in less secularised socio-cultural settings, religiosity may act as a stronger buffer against generalised social distrust, thus more effectively mitigating its negative impact on mental health. In contrast, our results show that in highly secularised societies, such as Slovenia, these effects are entirely absent.

Overall, our findings are consistent with social safety theory (Slavich, 2020, 2022; Slavich et al., 2022), which posits that the development and maintenance of positive

social bonds are essential to human well-being, and that threats to social safety – such as low trust in close relationships – act as significant psychological stressors, increasing vulnerability to mental health challenges. Interpreted through the lens of this theoretical framework, our results suggest that individuals rely on interpersonal relationships as a primary mechanism for coping with life stressors. Our findings emphasize that young people benefit significantly from having at least one high-trust resource, whether it is familial trust, generalised trust, or personal religiosity. The adaptability of these resources is a key insight. While the source of trust may change with age – shifting from family to broader social networks – and differ based on levels of religiosity, the presence of at least one strong source of trust is critical for maintaining mental health. Moreover, the compensatory role of religiosity highlights the importance of cultural context, where societal reinforcement of religious values can enhance its protective effects.

5.1. Implications

The findings of this study offer important implications for understanding the interplay between trust, religiosity, and mental health across age groups and cultural contexts. Our results suggest that low trust in close family members has a particularly strong negative impact on mental health during adolescence but becomes less significant as individuals age. This underscores the critical importance of fostering trust within family relationships during adolescence, a period of heightened vulnerability to mental health challenges. Interventions aimed at strengthening familial bonds may be especially effective for reducing depression, anxiety, and stress among younger individuals, while strategies for older age groups might focus on other sources of social or emotional support. Additionally, this study highlights the buffering role of religiosity in mitigating the mental health effects of low generalised trust. Individuals with higher personal religiosity appear to be less negatively impacted by low generalised trust, suggesting that religiosity provides psychological or social resources that alleviate the effects of a lack of trust in broader social networks. This finding is particularly significant in cultural contexts with high levels of religiosity, where religious practices and beliefs may play a key role in promoting resilience and well-being. Mental health interventions in these settings could benefit from incorporating or collaborating with religious frameworks to enhance their effectiveness. Overall, these findings emphasize the importance of considering developmental and cultural factors in mental health strategies. Targeted interventions that address age-specific vulnerabilities and leverage culturally relevant protective factors, such as religiosity, could significantly improve mental health outcomes across diverse populations.

5.2. Limitations and future research

Despite its valuable contributions, this study has several limitations. The primary limitation is its cross-sectional design, which prevents drawing causal inferences. Additionally, the use of self-reported measures may introduce bias. Another limitation lies in the study's focus on Croatia and Slovenia, which restricts the generalizability of the findings to other cultural contexts. Future research could employ longitudinal designs to explore how the relationships between generalised social trust, age, religiosity, and psychological well-being evolve throughout individuals' psychosocial development. Furthermore,

investigating these dynamics in diverse cultural settings would enhance understanding of how cultural values influence the interplay between trust and well-being. Specifically, based on our findings in Croatia and Slovenia, examining whether religiosity moderates the relationship between trust and psychological well-being more strongly in highly religious contexts using a larger sample of countries could provide valuable insights into the role of cultural religiosity in shaping these dynamics. Testing both of our hypotheses with alternative measures of psychological well-being or mental health would also strengthen the robustness of our findings, as it would allow us to determine whether the observed effects are consistent across different conceptualizations and measurements of well-being. Lastly, identifying other potential moderators or buffers against low social trust, beyond religiosity and age, would be beneficial for understanding a broader range of mechanisms that mitigate the negative psychological effects of weak trust.

6. Conclusion

In conclusion, this study offers important insights into how age and religiosity moderate the relationship between social trust and young people's mental health. The findings generally align with our theoretical expectations that the significance of trust in close family members diminishes as adolescents transition to adulthood. Additionally, the results show that religiosity may partly compensate for low generalised trust. More broadly, this study demonstrates how the negative psychological effects of low social trust can be alleviated through different mechanisms.

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Data availability statement

Data available upon request.

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